Weekly cetuximab + FOLFIRI

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 8 · weekly cetuximab only

Active medicines: Cetuximab

Emetogenic risk: Low

Medicine	Dose / route	When
Antihistamine	As prescribed	At least one hour before cetuximab Cetuximab premedication remains applicable.
Corticosteroid	As prescribed	At least one hour before cetuximab Reconcile low-risk antiemesis with this corticosteroid; do not duplicate steroids.
Metoclopramide	10 mg PO up to three times daily as required	Rescue only as prescribed No automatic delayed steroid course is carried over from Day 1 chemotherapy.

Additional phase-specific support

- Cetuximab only: no irinotecan, oxaliplatin, fluorouracil or atropine is scheduled on Day 8.
- Continue skin care, electrolyte monitoring, infusion precautions and the urgent contact plan.
- Low-risk antibody-only antiemesis is distinct from Day 1 moderate-risk chemotherapy prophylaxis.
- NCCP p1 permits cessation of observation after two tolerated cetuximab doses, whereas Erbitux SmPC section 4.2 requires monitoring during and for at least one hour after each infusion. Reconcile and document the local observation plan; do not promise early discharge.
- Folinic acid follows the bolus 5-FU hold/omission. Reconcile dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) inhibitors, DPD status and suspected fluoropyrimidine cardiotoxicity before administration.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00328 v7 · p6 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-11-07 · SACTCheck v0.83.8 · Regimen-level drafts; prescriber and oncology-pharmacy verification required